



STRATEGY & OPERATIONS

Vytalix Marketing And Content Engine

Vytalix company-build documentation · reference
14_MARKETING_AND_CONTENT_ENGINE

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

DATE

8 September 2026

VERSION

1.0 · Confidential draft

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Owner: Founder (acting CMO) · **Status:** PROPOSED · **Version:** 1.0 (7 September 2026) · **Governed by:**

/FACTS_BASE.md language rules and /docs/03_BRAND_SYSTEM.md voice rules · **Feeds:**
 /docs/13_SALES_ENGINE.md (lead generation), /assets/CONTENT_CALENDAR.csv ,
 /assets/LAUNCH_CAMPAGN.md

Executive view

- Vytalix has no website, LinkedIn page, content, list or press footprint (VERIFIED absence, FACTS_BASE \$1). Marketing therefore starts from zero and must produce sales conversations, not vanity reach: the only marketing KPI that matters in the first 90 days is **qualified conversations booked for the sales engine**.
- The strategy is founder-led, LinkedIn-first, evidence-led. One channel done daily (LinkedIn) beats six channels done badly. Website, newsletter and one research report are the supporting assets; paid media is not used before Month 6 (ESTIMATE budget < £500/month thereafter).
- Every piece of content obeys the stage-honest rule: Vytalix is pre-seed and founder-led; MaternaLink is "in development"; no claims of clients, partners, NHS approval or clinical outcomes. Credibility is built by showing thinking, not by claiming traction.
- Targets for the 90-day launch (PROPOSED, not forecasts): 1,500 relevant LinkedIn followers, 36 founder posts, 3 long-form articles, 1 report published, 1 webinar delivered, 300 newsletter subscribers, 15 inbound conversations, 5 inbound-sourced discovery calls.
- Cost: £0–£150/month tooling in the first 90 days (ESTIMATE: domain, email, newsletter tool, design tool); £2k–£4k one-off for the report design and any freelance editing (ESTIMATE, optional).

1. Marketing objectives (12 months, PROPOSED)

#	Objective	Why	12-month target (ESTIMATE)	Leading indicator
O1	Generate inbound sales conversations for Advisory/Consulting	Revenue is priority 1	60 inbound conversations; 20 discovery calls; 5 signed engagements from inbound	Profile views, post replies, form submissions
O2	Build investor visibility before the raise	Investors track founders for months before a round	300 investor-relevant followers; 10 investor inbound touches; report cited by 3 investors	Investor follows/engagement on posts
O3	Recruit design partners and clinical advisers for MaternaLink (in development)	Product credibility	3 clinical advisers; 2 design-partner LOIs	Clinician comments and DMs
O4	Establish Vytalix as a credible voice on digital maternity and UK–Africa health-tech	Credibility and PR	1 published report; 6 articles; 4 webinars; 3 earned media mentions	Article reads, citations, event invitations
O5	Seed the E-Learning waitlist	Year-2 revenue	500 waitlist sign-ups by Month 12	Newsletter growth

Rule: O1 gets ≥50% of marketing time in Months 1–6.

2. Audiences and content pillars

2.1 Audiences (aligned to sales segments S1–S6 in /docs/13_SALES_ENGINE.md)

Audience	What they need to believe	Content that earns it
Health-tech founders (S1)	"This person understands NHS/regulatory/go-to-market reality and can save me months"	Practical checklists, DTAC/MHRA explainers, honest founder diary
Investors and programme leads (S2)	"This founder is rigorous, honest and building something fundable"	Build-in-public updates, decision frameworks, market analysis
NHS/ICB/charity innovation leads (S3)	"They understand maternity services and inequality, and don't over-sell"	Maternity digital landscape analysis, MBRRACE-UK-cited commentary, service-design pieces
African ministries, NGOs, donors (S4)	"They respect what already exists in-country and design with partners"	Nigeria digital-maternity landscape, interoperability, programme-design notes
Clinicians (midwives, obstetricians)	"This product will not add clicks, will not diagnose, and listens"	Care-coordination pieces, escalation and handover topics, invitations to co-design
Learners (S6)	"These courses are practical"	Primers, templates, mini-lessons

2.2 Five content pillars

Pillar	Share of output	Example themes	Never
P1 Digital maternity and inequality	30%	Communication failures in maternity care; what a care-coordination layer must do; Black maternal health disparities (MBRRACE-UK cited)	Outcome claims for MaternaLink
P2 Health-tech go-to-market and regulation (UK)	25%	DTAC, DCB0129, DSPT, MHRA SaMD, procurement routes, pricing	Legal advice presented as certainty
P3 UK–Africa corridor	15%	Nigeria digital health landscape, NDPA, interoperability with existing programmes (e.g. mDoc in Ekiti/Lagos, VERIFIED public reporting), donor funding	Claims of government partnership
P4 Building Vytalix in public	20%	Decisions, mistakes, what we don't know, IP and naming lessons, the 90-day plan	Traction we don't have
P5 Learning and method	10%	Templates, checklists, frameworks (decision scoring, discovery interview guide)	"CPD-accredited"

3. Channel strategy (12 months)

Channel	Role	Cadence	Month live	Tooling (ESTIMATE cost)	Owner
Founder LinkedIn profile	Primary channel; all pillars	3 posts/week + 20 comments/week	Month 1 (Week 1)	£0	Founder
Vytalix LinkedIn company page	Repository and credibility; reposts founder content, publishes reports/roles	2 posts/week	Month 1 (Week 2, after name decision)	£0	Founder

Channel	Role	Cadence	Month live	Tooling (ESTIMATE cost)	Owner
Website (/website , static)	Conversion: services, MaternaLink (in development), report download, newsletter	Update monthly	Month 1 (Week 3, after incorporation details)	Domain £10–£15/yr; hosting £0 (static)	Founder
Newsletter ("Signal" — working title, TO VALIDATE)	Nurture; investor and clinician updates	Fortnightly	Month 2	£0–£30/month (e.g. Substack/Beehiiv free tier or MailerLite; TO VALIDATE)	Founder
Long-form articles (website + LinkedIn articles)	SEO and authority	1 per fortnight from Month 2	Month 2	£0	Founder
Research reports	Lead magnet, PR, investor evidence	2 per year	Month 3 (report 1), Month 9 (report 2)	£1k–£2k design each (ESTIMATE)	Founder + freelance designer
Webinars	Lead generation, clinician recruitment	Quarterly	Month 3	£0 (LinkedIn Live/Zoom free tier; TO VALIDATE)	Founder
Events	Relationship building	1 per fortnight (attend), 2 speaking slots/year	Month 1	Travel £50–£150/event (ESTIMATE)	Founder
PR / earned media	Credibility	Opportunistic; 1 pitch/month from Month 3	Month 3	£0	Founder
Video (short-form)	Repurposing; founder voice	2 per month from Month 4	Month 4	Phone + free editor	Founder
SEO	Long-term inbound	Continuous from website launch	Month 1	£0 (Google Search Console)	Founder
Email outbound (sales)	Owned by sales engine	See /docs/13_SALES_ENGINE.md	Month 1	CRM	Founder
Paid social	Test only	£300–£500/month test from Month 6 (ESTIMATE)	Month 6	Ads budget	Founder
Education (E-Learning primers)	Waitlist	Monthly primer from Month 6	Month 6	£0	Founder

3.1 LinkedIn operating rules

- Founder profile headline (PROPOSED): "Founder, Vytalix — health-technology advisory, consulting and MaternaLink (in development) · Digital maternity, UK and Nigeria".
- About section carries the stage disclosure: "Pre-seed, founder-led. MaternaLink is in development."
- Post format mix per week: 1 insight (P1/P2/P3), 1 build-in-public (P4), 1 practical asset (P5) or comment-led question.

- Posting window: Tuesday–Thursday 07:30–08:30 UK (TO VALIDATE against own analytics after 4 weeks).
- 20 substantive comments per week on posts by target-audience people (no "great post").
- No engagement bait, no emoji strings, no "🚀", no invented statistics; every number has a source in the post or first comment.

3.2 Website (static site in /website; content spec in /docs/03_BRAND_SYSTEM.md)

Pages that matter for conversion: Home; Advisory; Consulting (packages with prices marked "from"); Health Solutions (MaternaLink in development — intended-use statement visible); E-Learning (waitlist); Insights (articles, reports); About (stage-honest); Contact (form; where data goes). Legal pages per

/docs/15_LEGAL_AND_REGULATORY.md \$10.

3.3 SEO keyword clusters (targets, PROPOSED; volumes TO VALIDATE with a keyword tool)

Cluster	Primary keywords	Content that targets it	Intent
K1 NHS digital assurance	"DTAC checklist", "DCB0129 clinical safety case template", "DSPT for suppliers", "NHS digital health procurement"	Article A1, checklists, C6 sprint page	Commercial (\$1)
K2 Software as a medical device UK	"MHRA software as a medical device", "is my app a medical device UK", "UKCA class I software"	Article on intended use; P2 posts	Commercial (\$1)
K3 Digital maternity	"digital maternity care coordination", "maternity communication failures Ockenden", "maternal health app UK"	Report 1; P1 articles; Health Solutions page	Informational → S3
K4 Maternal health inequality	"Black maternal health UK statistics MBRRACE", "maternal mortality disparity UK"	Report 1; cited posts	Informational → credibility
K5 Nigeria digital health	"Nigeria digital health strategy", "NDPA compliance health app", "maternal health programmes Nigeria"	Report 2; P3 articles; C5 page	Commercial (\$4)
K6 Health-tech market entry	"how to sell to the NHS", "NHS market entry for health-tech", "UK health-tech go to market"	C4 page; Article A2	Commercial (\$1)
K7 Health-tech fundraising	"SEIS EIS health-tech", "Innovate UK health grants 2027", "investor readiness health-tech"	C7 page; P4 posts	Commercial (\$1)
K8 Brand	"Vytalix"	Every page	Navigational

Technical SEO minimum: titles/meta on every page; one H1; schema.org Organization; sitemap; Search Console verified by Week 3; page speed under 2 s (static site, no frameworks per brand system).

3.4 Events (UK and Africa; all dates TO VALIDATE against organiser sites before booking)

Event	Typical timing (TO VALIDATE)	Why attend	Target outcome
HETT Show, London	Autumn (Sept/Oct)	NHS digital buyers, vendors, HINs	10 conversations; 2 S3 leads
Digital Health Rewired, Birmingham	March	CNIOs, digital maternity leads	Speaking-slot application for 2028; 10 conversations
RCM / RCOG annual events	RCM autumn; RCOG World Congress (varies)	Midwifery and obstetric leadership; clinical advisers	2 clinical adviser conversations
HLTH Europe, Amsterdam	June	Investors, scale-ups	5 investor conversations

Event	Typical timing (TO VALIDATE)	Why attend	Target outcome
Africa health-tech gatherings (e.g. Africa Health ExCon; Lagos/Nairobi ecosystem events)	Varies	Ministries, NGOs, in-country partners	3 S4 conversations; in-country associate
Health Innovation Network / AHSN showcases (regional)	Rolling	Innovation leads, pilot routes	1 innovation-lead conversation/event
London health-tech founder meetups (various)	Monthly	S1 pipeline	5 founder conversations/month
Maternity voices / community maternal-health events	Rolling	Lived-experience input; community partners	1 community-partner relationship

Rule: one event per fortnight maximum; every event yields a same-day CRM update and a follow-up within 48 hours.

3.5 PR approach

- No press release before incorporation and a real announcement (e.g. report launch, adviser appointment with consent, first grant).
- Targets: Digital Health (UK trade), HTN, Med-Tech Innovation News, Nigeria Health Watch (guest analysis), TechCabal (Nigeria tech), local press for founder-story angles. All TO VALIDATE for current editorial contacts; use public editorial addresses only.
- Pitch format: 120-word email; the report finding as the hook; founder available for comment on maternity-digital topics (Ockenden/Kirkup follow-ups, MBRRACE-UK releases, NHS maternity digital announcements).

3.6 Email (nurture) and newsletter format

Newsletter working title: **"Signal"** (TO VALIDATE name clash; alternative "Vytalix Notes"). Fortnightly, Thursday 08:00 UK. 600–900 words.

Section	Content	Length
One thing	The single most useful insight of the fortnight (P1–P3)	150 words
What we're building	Honest MaternaLink (in development) and company update; one decision made	120 words
Three links, three sentences	Curated, cited	120 words
A template or tool	Downloadable (checklist, scoring sheet)	60 words + link
Ask	One request: an intro, a co-design volunteer, a critique	40 words
Footer	Stage disclosure; unsubscribe; legal entity details once incorporated; privacy link	—

Growth mechanics: every LinkedIn article ends with a newsletter link; report download requires email (with explicit consent wording per UK GDPR/PECR — see [/docs/15_LEGAL_AND_REGULATORY.md](#) §2.8); webinar registration double-opt in.

3.7 Video

From Month 4: 60–90-second founder-to-camera clips summarising the fortnight's insight; recorded in batches of four; captions burned in; no music; posted natively on LinkedIn. From Month 6: webinar recordings cut into 3–5 clips.

3.8 Education (E-Learning funnel)

Month 6+: monthly "10-minute primer" (P5) → waitlist for courses 2 and 4 in `/docs/01_BUSINESS_MODEL.md` \$6 (founder-authored, lowest clinical risk). No accreditation claims (business model \$6.3).

4. Twelve-month roadmap

Month	Marketing milestone (PROPOSED)	Sales linkage
1 (Sept–Oct 2026)	LinkedIn profile and page live; website live after incorporation; 12 posts; event 1	150-contact list; first 5 inbound conversations
2	Newsletter issue 1–2; articles A1–A2; webinar 1 announced	Inbound leads tagged in CRM
3	Report 1 published; webinar 1 delivered; PR pitch 1	Report used in every proposal appendix
4	Video launched; article A3; Search Console review	SEO leads tracked
5	Adviser announcements (only with consent); clinician co-design call	Design-partner LOIs
6	Paid test £300–£500; primer 1; e-learning waitlist opens	Waitlist → course pre-sales
7–9	Report 2 research; webinar 2 and 3; 2 speaking applications	Nigeria/NGO conversations
9	Report 2 published (Nigeria/UK–Africa corridor)	S4 pipeline
10–12	Webinar 4; year-end "what we learned" long-form; 2027 plan; e-learning course 2 launch content	Course sales; investor materials refreshed

5. Ninety-day launch campaign (week by week; Day 1 = Monday 14 September 2026)

Detailed daily version in `/assets/LAUNCH_CAMPAIGN.md` ; calendar in `/assets/CONTENT_CALEDNDAR.csv` .

Week	Dates	Theme	Actions	Output	KPI
1	14–20 Sept	Foundations	Rewrite LinkedIn profile; decide name (gate); reserve handles/domain; write posts 1–3; set up analytics sheet	Profile live; 3 posts	200 profile views
2	21–27 Sept	Company page and voice	Create company page (post name gate); posts 4–6; 20 comments; draft Article A1	Page live; 3 posts	100 page followers
3	28 Sept–4 Oct	Website launch	Publish website with entity details; Search Console; posts 7–9; newsletter tool set up	Site live; forms tested	300 site visits
4	5–11 Oct	Article A1 + first event	Publish A1 (DTAC); posts 10–12; attend event 1; newsletter issue 1	A1; issue 1	50 subscribers
5	12–18 Oct	Report 1 research	Interviews for Report 1 (10 of 20); posts 13–15; webinar 1 announced	Interview notes; registration page	30 webinar registrations
6	19–25 Oct	Article A2 + newsletter 2	Publish A2 (intended use); posts 16–18; issue 2; interviews 11–20	A2; issue 2	100 subscribers

Week	Dates	Theme	Actions	Output	KPI
7	26 Oct–1 Nov	Report drafting	Draft Report 1; posts 19–21; 1 PR pre-brief	Report draft v1	3 inbound conversations
8	2–8 Nov	Report design + webinar prep	Design; posts 22–24; issue 3; webinar rehearsal	Report v2; slides	60 registrations
9	9–15 Nov	Report 1 launch	Publish Report 1; launch post + carousel; PR pitch; posts 25–27	Report live; 1 media pitch	150 downloads in 14 days
10	16–22 Nov	Webinar 1	Deliver webinar; issue 4; posts 28–30; follow-up sequence	Recording; 5 clips queued	40 attendees; 5 discovery calls
11	23–29 Nov	Article A3 + clinician call	Publish A3; posts 31–33; co-design call for clinicians	A3; adviser conversations	3 clinician conversations
12	30 Nov–6 Dec	Consolidation	Issue 5; posts 34–36; analytics review; Q1 2027 plan	Review memo	Targets in \$9
13	7–13 Dec	Buffer / Day 90 review	Catch-up; case-study consent requests (if any engagement completed); 2027 calendar	2027 calendar	—

6. Twelve-week content calendar (titles are specific; all posts obey FACTS_BASE)

Week	Post 1 (Tue, insight)	Post 2 (Wed, build-in-public)	Post 3 (Thu, practical)	Long-form / other
1	"Why I'm starting a health-tech group with a consultancy, not an app"	"Day 1: what Vytalix is, what it isn't, and what I don't know yet"	"The three questions I ask before any digital maternity idea"	—
2	"Ockenden and Kirkup keep finding the same failure: communication" (cited)	"We are calling the AI risk score a research track. Here's why."	"A one-page intended-use statement — the most valuable document a health-tech founder can write"	Draft A1
3	"What DTAC actually asks for (and what it doesn't)"	"Naming a company when four similar names already exist"	"Discovery interview guide: 12 questions for maternity teams" (template)	Website live
4	"The £300-per-woman question: how to price a maternal health programme honestly"	"Week 4: first proposals out. What buyers pushed back on."	"DTAC readiness self-check: 20 items" (carousel)	A1: "DTAC without the panic"
5	"Ekiti already has a digital maternal programme. That's good news, if you design for it." (cited: Nigeria Health Watch)	"Interviewing 20 people for our first report: what surprised me in the first 10"	"Hazard log starter: 9 hazards every maternity communication tool must consider"	Webinar 1 announced
6	"Is your app a medical device? The one question that decides it (UK)"	"Why our v1 will display data and not interpret it"	"Grant calendar for maternal health-tech, Q1 2027 (TO VALIDATE dates)"	A2: "Intended use is a product decision"
7	"Black maternal health in the UK: the numbers, the sources, the caveats" (MBRRACE-UK cited)	"The IP conversation every founder with a collaborator must have early"	"How to run a 30-minute discovery call that respects a midwife's time"	Report drafting

Week	Post 1 (Tue, insight)	Post 2 (Wed, build-in-public)	Post 3 (Thu, practical)	Long-form / other
8	"Care coordination is not care. Why the distinction matters for safety."	"Report 1 is in design. Three findings I'm nervous to publish."	"Handover summary template (antenatal → intrapartum)"	Newsletter 3
9	"Report launch: The State of Digital Maternal Care Coordination — UK and Nigeria"	"What 20 interviews told us about who actually owns the communication problem"	"Ten things buyers of maternity digital tools should ask vendors" (carousel)	Report 1 live
10	"Webinar today: designing maternity communication tools that don't become medical devices by accident"	"What I got wrong in the first 60 days"	"Webinar recording + slide pack"	Webinar 1
11	"Interoperability with what's already there: designing for DHIS2 and incumbent platforms in Nigeria"	"We're looking for midwives and obstetricians to co-design MaternaLink (in development) — here's what it involves and what it doesn't"	"Clinical advisory group terms of reference (template)"	A3
12	"Selling to the NHS in 2027: the routes that actually work for a small vendor"	"90 days in: the scorecard, honestly"	"Our decision-scoring framework (10 criteria) — free template"	Newsletter 5; Q1 plan

7. Ten LinkedIn posts (founder voice; ready to publish; no fake claims)

Post 1 — Week 1, Tuesday. "Why I'm starting a health-tech group with a consultancy, not an app"

I'm starting Vytalix this week. Technology for Life. Most health-tech companies begin with an app and go looking for revenue. We're doing it the other way round. Vytalix has four parts: Advisory, Consulting, Health Solutions and E-Learning. The first two exist to pay for the second two. The product — MaternaLink, in development, a maternal care-coordination platform — will take years to validate properly. Services can be useful to someone next month. That's not a compromise. It's the only honest way I can see to build something that touches maternity care: earn the right to build by working inside the problem first. We're pre-seed and founder-led. No clients yet, no partners yet. I'll post what we learn as we go, including the things that don't work. If you run a health-tech company trying to reach the NHS, or a maternity service trying to make digital tools useful, I'd like to talk.

Post 2 — Week 1, Wednesday. "Day 1: what Vytalix is, what it isn't, and what I don't know yet"

Day 1 of building Vytalix in public. What it is: a UK health-technology group. Advisory and consulting for health-tech founders and health organisations. A product in development for maternity care coordination. Learning content later. What it isn't: NHS-approved, clinically validated, funded, or a team of twenty. It's me, a plan, and a list of people I need to convince. What I don't know yet: – whether the name survives a trademark search (there are four similar names out there) – whether buyers will pay what I think the work is worth – whether a maternity unit will co-design with a company this early I'll report back on all three. The plan is 90 days long and public.

Post 3 — Week 2, Tuesday. "Ockenden and Kirkup keep finding the same failure: communication"

Read the Ockenden report (2022) and the Kirkup report on East Kent (2022) side by side and one pattern repeats: information didn't move. Between shifts. Between community and hospital. Between the woman and the team.

Concerns raised and not acted on. Handovers that lost the thread. Both reports are public; the findings are theirs, not mine, and worth reading in full before anyone builds anything for maternity. Here's what I take from them as someone designing a care-coordination tool: the hardest problem is not prediction. It's making sure the right person sees the right thing in time, and that someone owns it. That's a workflow, ownership and accountability problem. Software can support it. Software can't replace the person who picks up the phone. We're designing MaternaLink (in development) around that view: display, route, remind, record who owns what. Not diagnose.

Post 4 — Week 2, Wednesday. "We are calling the AI risk score a research track. Here's why."

The original MaternaLink concept from 2025 described a multimodal AI score that would predict complications like pre-eclampsia and sepsis. It's an interesting research idea. It is not a product we can ship, and we're not going to pretend otherwise. To make a clinical prediction claim you need data you have lawful access to, an ethics-approved study, validation on the population you'll serve, a clinical safety case, and, in the UK, a medical-device pathway with the MHRA. We have none of that today. So v1 of MaternaLink is a care-coordination and communication platform. Non-diagnostic. It shows information to clinicians; it does not interpret it. The AI work moves to a research track that only advances with partners who can do it properly. Slower. Less exciting on a slide. Honest.

Post 5 — Week 3, Tuesday. "What DTAC actually asks for (and what it doesn't)"

If you want to sell digital health software to the NHS, someone will ask you about DTAC. Here's the shape of it, in plain terms. DTAC — Digital Technology Assessment Criteria — covers five things: clinical safety, data protection, technical security, interoperability, and usability/accessibility. It asks for evidence: a clinical safety case under DCB0129 with a named Clinical Safety Officer; your Data Security and Protection Toolkit status; a DPIA; Cyber Essentials (Plus is stronger); penetration test results; how you handle open standards; and accessibility conformance. What it doesn't do: it isn't a certification you "get" and put on your website. It's a set of criteria each buying organisation assesses. And it doesn't replace medical-device regulation if your software makes clinical claims. The founders who cope best treat it as product design input from day one, not a form to fill in at procurement. Source and detail: NHS England's DTAC page. This is a founder's summary, not legal advice.

Post 6 — Week 4, Tuesday. "The £300-per-woman question"

A number I've been thinking about: £300 per woman per year for a maternal digital-health programme in a Nigerian state. It's a figure I've seen proposed. Roughly a dollar a day per mother. It sounds reasonable until you compare it with how existing programmes are funded. The digital maternal programme already running in Ekiti and Lagos is donor-funded (MSD for Mothers, per public reporting from Nigeria Health Watch). Donor-funded per-woman costs for messaging-and-coordination programmes are, by my estimate, a small fraction of £300. I'd want to validate that with people who run those programmes before I quoted anything. My working rule for pricing anything in maternal health: start from cost-to-serve, add the evidence you're generating, and be able to defend the number in front of the person who signs off the budget and the person who benefits. If you've priced a programme like this, I'd like to compare notes.

Post 7 — Week 5, Tuesday. "Ekiti already has a digital maternal programme. That's good news, if you design for it."

When we looked at Ekiti State, Nigeria, we found something important: there's already a maternal digital-health programme running there — mDoc's Digital Mom Project, funded by MSD for Mothers, with around 24,000 women onboarded and 500+ providers trained as of January 2026 (Nigeria Health Watch reporting). A year ago that might

have looked like a problem. I think it's the opposite. It means the state has demand, digital literacy among providers, and a funder who cares. A new entrant that shows up promising to replace all that is going to lose. A new entrant that asks "what's the gap, and how do we connect to what exists?" has a conversation worth having. So our approach for Nigeria is: partner, complement, or pick a different state — never displace on price. Interoperability with what's there (DHIS2 exports, referral and facility-side coordination) is the design brief. If you work in this space in Nigeria, tell me what I'm missing.

Post 8 — Week 6, Tuesday. "Is your app a medical device? The one question that decides it (UK)"

Founders ask me this more than anything else. Here's the one question that decides most cases: Does your software interpret data about an individual for a medical purpose — diagnose, monitor, predict, prognose, treat? If yes, you're probably in medical-device territory under the UK Medical Devices Regulations 2002, and the MHRA's software-as-a-medical-device guidance applies. Classification, a technical file, and possibly an Approved Body follow. If no — you schedule, remind, educate, record, display, route messages — you're probably not. But write down why. A documented qualification decision is the thing a buyer, an investor and a regulator will all ask for. The line moves with features. A threshold alert on a blood-pressure reading crosses it. A "risk score" crosses it. A dashboard that shows the reading to a midwife doesn't. That's why we wrote MaternaLink's intended-use statement before we wrote code. It's one paragraph. It controls everything. Founder's summary, not legal advice. Get a regulatory adviser before you commit.

Post 9 — Week 7, Wednesday. "The IP conversation every founder with a collaborator must have early"

Uncomfortable truth from our first weeks: the most important document Vytalix needed wasn't a pitch deck. It was a written agreement about who owns what. If you've been building something with a collaborator — a co-founder, a developer, a friend who made the first prototype — and there's nothing in writing, you have a problem you can't see yet. Every investor will find it in the first ten minutes of diligence. Every serious customer will ask. What we're doing: a direct conversation, heads of terms in writing, then a solicitor. Options on the table are the normal ones — assignment for equity, a licence, or a joint venture. What matters is that it's written down and signed before any money, contract or pilot depends on it. It's awkward. It's also respect: for the work someone did, and for the company you're both trying to build. If you're in this situation, do it this week.

Post 10 — Week 12, Wednesday. "90 days in: the scorecard, honestly"

Ninety days ago I said I'd build Vytalix in public and report the numbers. Here they are. [Founder fills in actuals on Day 90 — do not publish targets as results.] Company: [incorporated / not yet]. Name: [cleared / changed]. IP: [agreed / in progress]. Sales: [n] conversations, [n] proposals, [n] signed. MaternaLink (in development): [scope decision], [n] clinician conversations, [n] design-partner conversations. Content: [n] posts, [n] articles, one report, one webinar, [n] subscribers. Investors: [n] conversations; no round opened. What worked: [two things]. What didn't: [two things]. What I'd tell someone starting today: [one thing]. Next 90 days: [three priorities]. Thank you to everyone who replied, argued, introduced, and corrected me. Keep doing it.

8. Three article outlines

A1 — "DTAC without the panic: a founder's map of NHS digital assurance" (1,800 words; Week 4; K1)

1. Why founders hear about DTAC too late (procurement moment vs. design moment).

2. The five domains, in plain English, with what evidence each expects.
3. Clinical safety: DCB0129 vs DCB0160, who the Clinical Safety Officer is, what a hazard log looks like (example rows).
4. Data protection: DSPT status levels, DPIA, ICO registration, lawful bases for health data.
5. Technical security: Cyber Essentials vs Plus, pen tests, what "reasonable" means at pre-seed.
6. Interoperability and accessibility: what to design for now, what can wait.
7. A 12-week readiness sequence for a pre-seed company (table).
8. Costs (ESTIMATE table with sources: IASME fees, ICO fee) and where to get qualified help. Disclaimer.

A2 — "Intended use is a product decision, not a legal afterthought" (1,500 words; Week 6; K2)

1. The paragraph that controls everything: the intended-use statement.
2. UK MDR 2002 qualification in one question; MHRA SaMD guidance; where the 2026 reform is heading (TO VALIDATE).
3. Worked example: MaternaLink (in development) v1 statement; what it allows and forbids.
4. Feature-by-feature line-crossing table (reminders, diary display, threshold alert, questionnaire scoring, risk score).
5. The commercial pay-off: shorter sales cycles, honest investor story, no forced re-classification.
6. How to document a qualification decision (template).
7. Disclaimer and when to hire a regulatory consultant.

A3 — "Designing for what already exists: digital maternal health in Nigeria in 2026" (2,000 words; Week 11; K5)

1. What's already there: national frameworks, DHIS2, existing programmes (mDoc/MSD for Mothers in Lagos and Ekiti; cited).
2. Why "replace" fails and "connect" wins: budgets, donor logic, provider workload.
3. Three entry models for a new platform: interoperability layer; facility-side coordination; NGO-funded deployment in an uncovered LGA or state.
4. Regulatory and data obligations (NDPA 2023 registration thresholds, NHREC ethics for evaluations, NAFDAC position on software — all TO VALIDATE).
5. Pricing honestly: cost-to-serve per enrolled woman; who actually pays.
6. What we're doing about it (MaternaLink in development; the Ekiti conversation as a partnership question, not a sales question).
7. Questions for people in the field.

9. Two research report concepts (with methodology)

Report 1 — "The State of Digital Maternal Care Coordination: UK and Nigeria, 2026" (Month 3; 20–28 pages)

- **Research question:** Where does information fail to move in maternity care, and what do frontline professionals and women say a digital coordination layer must and must not do?

- **Method (mixed, small-n, transparent):** (a) desk review of public inquiries (Ockenden, Kirkup, MBRRACE-UK), NHS maternity digital policy and Nigeria national maternal-health frameworks; (b) 20 semi-structured interviews (10 UK: midwives, obstetricians, maternity digital leads, charity staff; 10 Nigeria: clinicians, programme managers, NGO staff), recruited via LinkedIn and warm intros, 30 minutes each, consented, anonymised; (c) a 15-question online survey (target $n \geq 60$; convenience sample, stated as such); (d) landscape scan of 15–20 existing tools with public information only.
- **Ethics and data:** not research involving patients; no patient data; participant information sheet and consent; data stored in Vytalix-controlled workspace; ICO registration in place before survey launch; privacy notice linked. TO VALIDATE whether any NHS staff participation needs local approval (usually not for professional opinion surveys, but confirm).
- **Outputs:** 8–10 findings; a "must / must not" design list; a buyer's checklist; a stated limitations section; open dataset of aggregated survey results.
- **Distribution:** gated download; LinkedIn carousel; PR pitch; sent to all interviewees; used in proposals and investor materials.
- **KPIs:** 150 downloads in 14 days; 400 in 90 days; 3 citations; 5 inbound conversations.

Report 2 — "Paying for Maternal Digital Health in Africa: cost-to-serve, funders and what governments actually buy" (Month 9; 20–30 pages)

- **Research question:** What does it really cost to serve one enrolled woman per year with a messaging-and-coordination programme, who pays today, and what evidence do funders and ministries require?
- **Method:** (a) public programme documentation and evaluations (mDoc, MomConnect-style programmes, GFF/MSD for Mothers reports; all cited); (b) 15 interviews (donor programme officers, state MoH digital leads, NGO implementers, telecom health leads) in Nigeria, Kenya and one further market; (c) a bottom-up cost model with stated assumptions (SMS/WhatsApp costs, CHW time, hosting, support) published as a spreadsheet; (d) sensitivity analysis.
- **Ethics and data:** professional interviews only; consented; no patient data.
- **Outputs:** cost-per-woman-year ranges by delivery model; funder evidence requirements matrix; a pricing playbook; explicit note that Vytalix's own proposals will follow the report's method.
- **KPIs:** 300 downloads in 90 days; 3 S4 conversations; 1 donor conversation; 1 media mention.

10. Webinar plan (Webinar 1, Week 10 — Tuesday 17 November 2026, 12:30–13:15 UK; TO VALIDATE against founder diary)

Element	Detail
Title	"Designing maternity communication tools that don't become medical devices by accident"
Audience	Health-tech founders and product leads (S1); maternity digital leads (S3); clinicians curious about co-design
Format	25-minute talk + 15-minute Q&A + 5-minute close; LinkedIn Live or Zoom (TO VALIDATE tooling); recorded
Speakers	Founder; one guest (a clinical or regulatory adviser) only if confirmed in writing ; otherwise founder-only
Agenda	1. The communication-failure pattern (cited) · 2. The intended-use statement · 3. Feature line-crossing table · 4. DTAC in 5 minutes · 5. Report 1 headline findings · 6. Q&A · 7. Offer: free 30-minute session (sales engine rule: one per prospect)
Promotion	Announced Week 5; 6 posts; newsletter issues 2–4; direct invites to 100 CRM contacts; company page event

Element	Detail
Targets	60 registrations; 40% attendance (24+); 5 discovery calls booked; 10 newsletter sign-ups
Follow-up	Same-day recording email; 3-touch sequence (see /docs/13_SALES_ENGINE.md §8); clips for video channel
Compliance	Registration privacy notice; consent for marketing emails; no recording of attendee video; disclaimers (not legal/regulatory advice)

11. Marketing KPIs (monthly review in [/docs/17_FOUNDER_OPERATING_SYSTEM.md](#))

KPI	Formula	Month 1 target	Month 2	Month 3	Month 12
Posts published	count	12	12	12	140
Relevant followers (founder + page)	count (ICP-tagged sample)	500	900	1,500	5,000
Post engagement rate	(reactions + comments + shares) / impressions	3%	4%	5%	5%
Inbound conversations	DMs/emails/forms from non-outbound contacts	3	6	15 (cum.)	60
Inbound → discovery call	discovery calls from inbound / inbound conversations	25%	30%	33%	33%
Newsletter subscribers	count	0	100	300	1,500
Newsletter open rate	opens / delivered	—	45%	45%	40%
Website sessions	monthly	300	800	1,500	5,000
Website conversion	(forms + downloads + sign-ups) / sessions	2%	3%	4%	4%
Report downloads	count	—	—	150	700 (both reports)
Webinar registrations / attendance	count / %	—	30	60 / 40%	4 webinars, 250 registrations
Earned media mentions	count	0	0	1	6
Marketing-sourced revenue	£ signed from inbound-sourced deals	£0	£0	£5k (ESTIMATE)	£40k (ESTIMATE)
Cost per inbound conversation	marketing spend / inbound conversations	< £20	< £20	< £20	< £50

Priorities / Risks / Next actions

Priorities

1. Founder LinkedIn live with stage-honest profile and posts 1–3 in Week 1; company page and website follow the name and incorporation gates.
2. Report 1 as the credibility anchor by Week 9; webinar 1 in Week 10.

3. Every asset routed into the CRM: inbound tagged, followed up within 48 hours.

Risks

Risk	Likelihood	Impact	Mitigation
Founder time: content displaces selling	High	High	Content batched Friday afternoons (2 hours); 3 posts/week cap; sales first
Over-claiming slips into posts	Medium	High	FACTS_BASE checklist before publishing; no numbers without sources
Name change after launch	Medium	Medium	Brand system designed for a rename; no paid brand assets before clearance
Report interviews slow to recruit	Medium	Medium	Start recruitment Week 3; accept n = 15 minimum, state limitations
Newsletter/consent non-compliance (PECR)	Low	Medium	Double opt-in; privacy notice; ICO registration before data capture

Next actions

Action	Owner	Deadline
Rewrite LinkedIn profile; publish Post 1	Founder	Tue 15 Sept 2026
Reserve domain and handles for chosen name (post clearance)	Founder	Fri 18 Sept 2026
Set up <code>/assets/CONTENT_CALENDAR.csv</code> tracking and analytics sheet	Founder	Fri 18 Sept 2026
Newsletter tool selected; privacy wording approved	Founder	Fri 2 Oct 2026
Report 1 interview list (30 names) and consent form	Founder	Fri 9 Oct 2026